
DEXTAL INJECTION 50%W/V (10ML AMPOULE)

COMPOSITION: Each ampoule contains Dextrose Monohydrate 50% w/v.

DESCRIPTION: Colourless to faintly yellow colour sterile solution.

PHARMACODYNAMICS:

Glucose provides energy. Glucose when given intravenously with water metabolises into glycogen leaving water to be absorbed to correct fluid imbalance. Glucose increases the movements of electrolytes across the semi-permeable membrane.

PHARMACOKINETICS:

Glucose is rapidly absorbed from the gastrointestinal tract. It is metabolized to carbon dioxide and water with the release of approx. 4 calories/gram of energy. It is readily metabolized. It may decelerate body protein and nitrogen losses, promote glycogen deposition and decrease/prevent ketosis in sufficient dose.

Bioavailability: As the solution is directly administered into the blood, it is 100% available.

INDICATIONS:

DEXTAL INJECTION 50% W/V is indicated for the treatment of hypoglycaemia coma. It is also for parenteral nutrition and high caloric carbohydrate therapy, especially when fluid intake is limited.

RECOMMENDED DOSAGE:

Adult patients

According to individual requirements:

Dextrose 50%: Up to 14mL/kg of body weight and per day
Up to 1mL/kg body weight and per hour

Pediatric patients

Mean requirements/kg of body weight and per day:

1 st year of life	: 8 – 15g dextrose
2 nd year of life	: 12 – 15g dextrose
3 rd – 5 th year of life	: 12g dextrose
6 th – 10 th year of life	: 10g dextrose

MODE OF ADMINISTRATION: Intravenous (IV)

Glucose solution with a concentration greater than 5% are hypertonic and should be administered by slow intravenous infusion via a central vein.

CONTRAINDICATIONS:

Dextrose monohydrate is contraindicated in patients with:

- Diabetes mellitus (with the exception of hypoglycaemia conditions)
- Glucose intolerance, hypotonic dehydration if lacking electrolytes are not replaced
- Over hydration
- Hypokalemia
- Hyperosmolar coma
- Acidosis

WARNINGS AND PRECAUTIONS:

Blood glucose, serum electrolytes and water balance should be monitored regularly. Electrolytes are to be supplemented as required. The compatibility of any additives to this solution should be checked before use.

Dextrose solutions should not be administered through the same infusion set through which also blood has or may be given because of the risk of pseudo agglutination.

Single dose container. Discard all unused contents. Do not use if leakage is detected. If any visible solid particles, growth or turbidity appears during storage, the product should not be used.

INTERACTION WITH OTHER MEDICAMENTS:

Both the glycaemic effects of intravenous glucose and its effects on water and electrolyte balance should be taken into account when using intravenous glucose in patients treated with other substances that affect glycaemic control, or fluid and/or electrolyte balance.

PREGNANCY AND LACTATION:

Intrapartum maternal intravenous glucose infusion may result in foetal insulin production, with an associated risk of foetal hyperglycaemia and metabolic acidosis as well as rebound hyperglycaemia in the neonate.

Pregnancy

Glucose solution can be used during pregnancy. However, caution should be exercised when glucose solution is used intrapartum.

Fertility

There are no adequate data of the effect of Glucose on fertility.

Lactation

There are no adequate data of using Glucose solution during lactation. Glucose solutions have been used during lactation.

SIDE EFFECTS:

Dextrose injection and particularly hypertonic dextrose injection may have low pH, and such solution when injected may irritate the venous intima near the site of the injection, causing thrombophlebitis.

Hyperglycaemia and renal losses may occur in case of reduced glucose tolerance. These manifestations are normally prevented by reducing the dosage and/or giving insulin.

Enhanced bilirubin and lactate levels may be found if the recommended dosage is exceeded.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Dextrose solution intravenous can cause fluid overload resulting in dilution of serum electrolyte concentrations, overhydration, congested states or pulmonary oedema.

Hyperglycaemia and glucosuria may be functions of rate of administration or metabolic insufficiency. To minimize these condition, slow infusion rate, monitor blood and urine glucose; if necessary, stop administration of dextrose or/and administer insulin.

STORAGE CONDITIONS:

Store below 30°C. Keep out of reach of children. *Jauhkan daripada kanak-kanak.*

SHELF LIFE:

3 years.

PACK SIZES:

10's x 10ml ampoules.

PRODUCT REGISTRATION HOLDER & MANUFACTURER:**DUOPHARMA (M) SDN. BHD.**

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